

AXINOL® 20 MG TABLET

Composition

Red film-coated round bi-convex tablets, embossed with 'T' on one side.
Each tablet contains trimetazidine dihydrochloride 20 mg.

Properties

Trimetazidine is an antianginal and ant ischaemic agent which differs chemically and pharmacologically from beta-blocking agents, nitrates, and calcium antagonists. Animal and human studies have suggested cardio protective effects of trimetazidine during ischaemic states.

Trimetazidine maintains the level of energy of the cells during systemic hypoxemia or local ischaemia. It prevents electrophysiological manifestations of ischaemia. It diminishes the rise in the ST segment in ECG of dogs with experimental myocardial infarction.

Studies in animals under severe hypoxia show that trimetazidine prevents the collapse in the levels of energy supplying ATP and cyclic AMP stores in the cerebral cells. Trimetazidine maintains functional activity of mitochondrial enzymes, the site of cellular energy production in the liver cell. The drug has been shown to protect tissues against oxygen-free radicals induced by chemicals or during ischaemic reperfusion.

Controlled double-blind trials in angina have demonstrated that trimetazidine leads to increased exercise tolerance beginning on the 15th day of treatment, rise in total work capacity in the exercise tolerance test, decreased the frequency and intensity of anginal episodes, and decreased trinitrate consumption.

Trimetazidine is rapidly absorbed following oral administration, with a plasma peak level of 90 - 130 ng/ml attained 2 - 3 hours after administration of 40mg doses.

The apparent volume of distribution is 4.8 L/kg; protein binding approximately 21%. Elimination of trimetazidine is primarily urinary; the mean half-life is 5.5 hours.

Indications

Axinol is indicated in adults as add-on therapy for the symptomatic treatment of patients with stable angina pectoris who are inadequately controlled by or intolerant to first-line antianginal therapies.

Contraindications

Axinol is contraindicated in patients with:

- Hypersensitivity to trimetazidine
- Parkinson disease, parkinsonian symptoms, tremors, restless leg syndrome, and other related movement disorders.
- Severe renal impairment (creatinine clearance <30ml/min)

Side Effects

Gastrointestinal: Gastric burning and other gastrointestinal disturbances have been described occasionally during oral trimetazidine therapy.

Cardiovascular: Most studies have reported no effect of oral trimetazidine, given acutely or chronically, on blood pressure or heart rate. There are however some isolated reports of increase in mean arterial pressure or worsening of hypertension in some patients.

Nervous system disorders (frequency not known): Parkinsonian symptoms (tremor, akinesia, hypertonia), gait, instability, restless leg syndrome, other related movement disorders usually reversible after treatment discontinuation.

Precautions/ Warnings

Caution should be exercised in renal or hepatic insufficient patients due to lack of pharmacokinetics studies in these patients.

Although heart rate and blood pressure were unaffected in most studies, there have been a few reports of hypertensive effect of trimetazidine. The drug should be used with caution in hypertensive patients and those with unstable angina.

Trimetazidine can cause or worsen parkinsonian symptoms (tremor, akinesia, hypertonia), which should be regularly investigated, especially in elderly patients. In doubtful cases, patients should be referred to a neurologist for appropriate investigations.

The occurrence of movement disorders such as parkinsonian symptoms, restless leg syndrome, tremors, gait instability should lead to definitive withdrawal of trimetazidine.

These cases have a low incidence and are usually reversible after treatment discontinuation. The majority of the patients recovered within 4 months after trimetazidine withdrawal. If parkinsonian symptoms persist more than 4 months after drug discontinuation, a neurologist opinion should be sought.

Falls may occur, related to gait instability or hypotension, in particular in patients taking antihypertensive treatment.

Caution should be exercised when prescribing trimetazidine to patients in whom an increased exposure is expected:

- moderate renal impairment,
- elderly patients older than 75 years old.

Until the apparent lack of toxicity of trimetazidine has been confirmed in long term studies, complete blood counts, hepatic function tests and renal function tests should be monitored periodically during therapy.

Use in Pregnancy and Lactation:

Safety of use of trimetazidine in pregnant and nursing mothers has not been established.

Use in children:

Safety of use of trimetazidine in children has not been established.

Drug Interactions

Since non-interaction with MAO inhibitors has not been established, trimetazidine should not be co-administered with these agents. No other drug interactions have been reported.

Dosage and Administration

Treatment of chronic stable angina: 20 mg three times daily. The tablets should be taken with meals.

The benefit of the treatment should be assessed after three months and trimetazidine should be discontinued if there is no treatment response.

Special populations

Patients with renal impairment:

In patients with moderate renal impairment (creatinine clearance 30-60 ml/min), the recommended dose is 1 tablet of 20mg twice daily, i.e., one in the morning and one in the evening during meals.

Elderly patients

Elderly patients may have increased trimetazidine exposure due to age-related decrease in renal function. In patients with moderate renal impairment (creatinine clearance 30-60 ml/min), the recommended dose is 1 tablet of 20mg twice daily, i.e., one in the morning and one in the evening during meals. Dose titration in elderly patients should be exercised with caution.

Overdosage

In overdosage, patients should be monitored closely in institutions with cardiac care facilities. Use of appropriate supportive measures in addition to gastric lavage are recommended.

Packing

Box of (9 x 10), (10 x 10) and (50 x 10) tablets.

Storage

Store in a cool dry place, below 30°C

Protect from light.

Keep out of reach of children.

Jauhi daripada kanak-kanak.

Shelf Life: Please refer to outer box label

Manufactured by:

T.O. Chemicals (1979) Ltd.

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Date of revision: 9 Jan 2023